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College of Health Science

School of Public health

Health Service Management Group Assignment

TOPIC: Historical Development of Health Sector Development Program II

Team-3

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1. Introduction

A twenty-year Health Sector Development Programme (HSDP) was drafted in EFY 1986/87 with a rolling five-year program period. The program's main objectives were to build basic infrastructure, provide standard facilities and supplies, and develop and deploy appropriate human resources for equitable health care delivery. The total budget for the 20-year period was estimated to be 20 billion Birr, with the first five years requiring 5 billion Birr.

The first five-year phase of this plan, HSDP I, ran from 1997/98 to 2001/02 and had the following objectives:

- Increase access to health care from 40% to 50-55%.
- Improve service quality by providing training and a better supply of inputs.
- Strengthen the management of health services at both federal and regional levels.
- Encourage private sector and NGO participation by creating an environment that enables participation, coordination, and fund mobilization.

The achievements, constraints, and lessons learned from HSDP I was documented in the Mid Term Review (MTR) in March 2001 and the Final Evaluation of HSDP I in March 2003. The Programme Implementation Manual (PIM), published in October 1998, outlined the governance and management arrangements for all stakeholders and the performance indicators for monitoring the program.

Based on recommendations from the MTR, a three-year Health Sector Development Programme (HSDP II) was prepared, covering the period of EFY 1995-1997. This duration allowed for alignment with the Second National Development Plan (NDP II) and the Sustainable Development and Poverty Reduction Programme (SDPRP) of the Government of Ethiopia (GOE). HSDP II was approved at ARM 4 in 2002 and formally reviewed twice as part of JRM 3 in March 2003 and JRM 4 in June 2004. Similar to HSDP I, HSDP II was composed of eight components that comprehensively defined the health sector. Each component had detailed objectives, targets, strategies, and indicators. The total three-year budget for HSDP II was 6.8 billion Birr. The budget for each component was estimated as follows:

Table 1. Health components of HSDP II with their budgets

HEALTH COMPONENTS HSDP II	BUDGET (Birr)	PERCENT
1. Service Delivery and Quality of Care	3,924,851	57
2. Health Facility Rehabilitation and Expansion	1,609,119	23
3. Human Resource Development	440,983	6.5
4. Pharmaceutical Supply and Management of ED	677,417	10
5. Information, Education and Communication (IEC)	84,934	1
6. Support and Advisory Services*	577,808	8.5
TOTAL 3 YEAR BUDGET HSDP II 1995-1997	6,868,232	
Capital	3,168,000	46
Recurrent	4,000,000	54

* This included health management and information systems, health care financing, monitoring and evaluation and cross cutting issues.

Of the total 6.8 billion Birr budgeted from Federal resources for HSDP II, 54% was allocated for recurrent expenditure (including salaries) and 46% for capital expenditure. Approximately 93% of the budget was designated for the regions based on population size, existing infrastructure, and revenue generation capacity. The remaining 7% was to be used by the Federal Ministry of Health (FMOH).

2. Service Delivery during HSDP II

Ethiopia has implemented fundamental health reform measures to tackle widespread health problems and address the high demand for healthcare in rural areas beginning from 1991. The Health Sector Development Programme (HSDP) was initiated in 1998 with the goal of improving the coverage and quality of health services. In line with the Millennium Development Goals (MDGs), the government introduced two key initiatives:

- The Health Service Extension Programme (HSEP) and
- The Accelerated Expansion of Primary Health Care Coverage (AEPHCC).

The HSDP II component of service delivery covers various sub-components of health service delivery, including communicable disease control, family health services, and cross-cutting issues like gender and pastoralist health care.

Table 2. Relevant MDG indicators within HSDP II

Indicator relevant for the MDGs	Baseline 1994 HSDP II	Status 1997 End HSDP II
% Births attended by skilled staff	10%	12%
Contraceptive acceptor rate	14%	21%
%<1 yr immunised for DPT3	51%	70%
%<1 yr immunised for measles	42%	61%
TB cure rate	66%	65%
Outpatients visits/pp/yr	0.27 (1993)	0.30
GOE per capita expenditure on health (Birr/pp in public sector)	NA	16.3

Source: HMIS data base 1996-1997 to 2004-2005 prepared during HSDP II evaluation

Health Services Extension Programme (HSEP)

The HSEP is a community-based primary healthcare service that provides a package of preventive, promotive, and curative services, with a specific focus on mothers and children. The program has been implemented in nearly all regions and is considered by many as one of the best programs Ethiopia has undertaken in recent years. It is delivered by female Health Extension Workers (HEWs) who undergo a one-year training and are formally employed and salaried by their respective Woredas. The HSEP aimed to increase access to essential health interventions and improve health awareness.

Despite its achievements, the HSEP faced several constraints. These include inadequate resources for training, such as a shortage of trainers and appropriate teaching materials, as well as problems with the selection and recruitment of HEWs. The deployment of HEWs is also hindered by a lack of synchronization between the construction of health posts and the completion of training. Other issues include weak supportive supervision, unclear roles and responsibilities between HEWs and other health workers, and a mismatch between community expectations and the services provided by the HEWs, who are expected to provide curative services that are not always included in their package.

Communicable Disease Prevention and Control

The HSDP II was also put significant efforts in controlling major communicable diseases:

- **Malaria:** Malaria is the leading public health problem in Ethiopia, with 68% of the population at risk. Achievements during HSDP II include securing significant funding from the Global Fund for AIDS, Tuberculosis, and Malaria (GFATM) and revising treatment guidelines to use Artemether-Lumefantrine (Coartem) as the first-line treatment for uncomplicated falciparum malaria. The program also made strides in distributing insecticide-treated nets (ITNs), with household coverage increasing to 23% in mid-1998. But Challenges also persisted, such as the high cost of Coartem and shortages of Rapid Diagnostic Tests (RDTs).
- **Tuberculosis and Leprosy:** The program aimed to increase tuberculosis (TB) control service coverage from 50% to 75%. Progress has been made in geographic expansion, with DOTS (Directly Observed Treatment, Short-course) Woreda coverage increasing from 50% to 90%. The program's treatment success rate reached 81% in 1997, close to the 85% target. However, case detection remains a major challenge, falling far below the 70% target. The prevalence of leprosy has also been reduced.
- **HIV/AIDS:** The HSDP II aimed to reduce morbidity, disability, and mortality from HIV/AIDS. The program saw progress in developing strategic documents, establishing over 650 VCT (Voluntary Counseling and Testing) sites, and integrating PMTCT (Prevention of Mother-to-Child Transmission) in 129 centers. Despite these achievements, VCT services, especially in rural areas, are constrained by a lack of trained counselors, technicians, and equipment. It was noted that care and support programs are lagging behind in quality and quantity.

Family Health Services and Medical Services

The HSDP II aimed to provide integrated family health services and improve medical service delivery. It was found that while potential health service coverage increased from 61% to 72%

during HSDP II, the utilization of health services did not. This discrepancy was attributed to staff shortages, temporary drug supply issues, and low incidence of diseases like malaria.

Challenges in medical services include a critical shortage of skilled health personnel, inadequate equipment and supplies, and a lack of a clear strategy for quality assurance. Many health facilities, particularly in rural areas, were not operating at their full potential and some lack basic infrastructure like water and power supply.

Cross-Cutting Issues

Other issues that affected all aspects of health service delivery were:

- **Gender**: Despite a National Policy on Ethiopian Women, little progress has been made in integrating gender issues into health planning and programming. It was found that a lack of genuine commitment, insufficient training on gender mainstreaming, and a failure to establish women's affairs units in various health structures.
- **Pastoralist Health Care**: The HSDP II acknowledged the lack of a clearly defined strategy for pastoralist regions, which made up 11% of the population. Achievements include the development of a program for accelerated development and a specific HEW training package for pastoralist areas. However, the actual establishment of a health delivery system compatible with the lifestyle of nomadic pastoralists has not yet been fully achieved.

The HSDP II has made notable progress in expanding health service programs like the HSEP and improving the control of communicable diseases. However, significant challenges remained, including resource constraints, staff shortages, and the need for better synchronization between training and deployment.

3. Support Services under HSDP II

1. Facility Construction and Rehabilitation

The objective of this component was to improve access to and the quality of health services by expanding existing health facilities and building new ones. Key strategies included constructing new health facilities, particularly Primary Health Care Units (PHCUs), in underserved areas and developing national standards for all health facilities. The goal was to increase potential health service coverage from 62% to 72% and to equip PHCUs according to these standards.

Significant achievements were made in this area, especially at the lower levels of the health system, which aligns with the government's focus on improving primary-level health access. The number of health posts increased by 2,901, from 1,311 to 4,212. Health centers also saw a notable increase of 192, surpassing the HSDP target of 172. While the public sector constructed three new hospitals against a target of six, an additional 33 new hospitals were built by NGOs and the private sector, bringing the total increase in hospitals to 36.

Furthermore, new, rehabilitated, and refurbished facilities were supplied with medical equipment that met FMOH standards. Some regions, such as Dire Dawa and Benishangul/Gumuz, have managed to keep almost 90% of their medical equipment functional by outsourcing regular maintenance. The push to construct health facilities near boreholes was a positive development that would help ensure a clean water supply. Health facilities were also becoming more patient-friendly with better labeling and patient flow, largely due to the implementation of the Civil Service Reform Programme. The FMOH had also begun a facility mapping initiative to create a database of health facilities to guide future investments and promote equitable distribution.

Despite this progress, several constraints and challenges exist. There remained a significant gap to fill, as health facilities were still limited in number and unevenly distributed. The strategy to rehabilitate existing facilities had not been fully implemented. A lack of synchronized planning has led to some facilities being completed without arrangements for staff, furniture, medical equipment, or water supply. The available medical equipment is not fully utilized due to a shortage of qualified health personnel. Additionally, the budget for supervision, contract administration, and maintenance was inadequate, leading to poor physical conditions of health facilities and problems with maintaining furniture and equipment at all levels.

2. Human Resource Development

The main goal of the human resource development component of HSDP II was to train and provide an adequate number of qualified health workers. Key objectives included supplying skilled staff to new facilities, improving the capacity of existing workers, strengthening continuing education and in-service training, and revising curricula for certain health worker categories.

A major achievement was the development of important policy documents, including the Health Sector Human Resource Development Framework (2006-2010) and the Essential Health Services Package. The Health Services and Training Department was also restructured, leading

to the creation of the Human Resources Development Department. The country's technical workforce, at 45,859, was very small for a population of over 70 million, resulting in some of the lowest health professional-to-population ratios in the world. The doctor-to-population ratio, for instance, worsened between 1994 and 1997 EC.

The workforce composition also has internal distortions. A considerable portion (over 40%) consists of low-trained staff, such as Front-Line Health Workers (FLHW), Health Assistants (HA), and Health Extension Workers (HEW), while highly skilled medical doctors and Health Officers make up only 2% and 5% respectively. There was an alarming shortage of midwives, and about half of them were male, which could deter women from seeking care for obstetric conditions. An acute shortage of emergency obstetric skills among General Practitioners was also noted during regional visits.

One significant constraint is the uneven distribution of health staff, with most professionals concentrated in urban areas. The public sector has lost a large number of physicians to the private sector, with the proportion of medical doctors in public institutions decreasing from 73% in 1994 EC to 44% in 1997 EC. Another major issue was that human resource development has lagged behind network planning and construction, leading to new health facilities remaining unstaffed for long periods after completion. There is also a lack of effective incentive packages to encourage staff deployment to rural areas, and staff turnover is high across all regions.

3. Pharmaceutical Supply and Management

The overall goal of the pharmaceutical sector was to ensure and sustain the availability of safe, effective, and quality drugs at an affordable price and to promote their rational use. HSDP II targets included making at least 80% of Essential Drugs (ED) available in each health facility, reducing the number of drugs per prescription, and increasing generic prescriptions.

In terms of policy, new pharmaceutical legislation had been prepared, and a policy on the supply and use of anti-retroviral drugs had been developed and distributed. However, these policy documents and other important manuals, like standard treatment guidelines, were often only available at the regional level and not at the zonal or woreda levels.

The budget for pharmaceuticals has increased over the years, with donor contributions playing a significant role, especially for vertical health programs like Family Planning. However, the budget remained insufficient to meet the growing demand for services. Delays in the transfer of drug budgets, receipt of donated drugs close to or after their expiry dates, and a lack of integrated budgetary planning at the regional level were common constraints.

Procurement and logistics management faced several challenges. Transport limitations often hinder the timely distribution of essential drugs and equipment. There were also regular stock-

outs of drugs at PHARMID, and the capacity of PASS to handle procurement functions is insufficient, leading to delays. The decentralization of procurement to the Woreda level might have been too early. Furthermore, drugs for vertical programs like anti-malarial and family planning are often selected and quantified without considering the specific disease burdens of each region, which led to inadequate supplies in some facilities and overstocks in others.

Storage facilities were frequently found to be inadequate, lacking mechanisms to monitor temperature. Expired drugs were also found in many places. A proper Drug Management Information System was largely non-existent, making monitoring difficult. While there was a good availability of drugs (95-100%) in special pharmacies, regular stock-outs are common in budget pharmacies, with the special pharmacies often compensating for these shortages.

Regarding Rational Drug Use (RDU), the average number of drugs prescribed per encounter was 1.9, which was close to the HSDP II target of 1.84. However, the use of antibiotics (58%) was too high compared to the target of about 20%, and the percentage of injections used was 23%, which was higher than the accepted norm of less than 15%. The absence of Standard Treatment Guidelines and the National Drug Formulary in most health facilities also hinders RDU.

Overall, while there had been improvements, more effort was needed in human resource availability, procurement and distribution, rational drug use, and budget allocation to address the persistent issues in the pharmaceutical sector.

4. Finance and Governance in HSDP II

Financing the Sector: Challenges and Under-funding

A central issue identified was the significant under-funding of the health sector during HSDP II. The actual health expenditures amounted to only 43% of the estimated program costs, and the consolidated health budget was just 56% of the expected amount. The government's health budget, as a percentage of total public expenditure, followed a declining trend, falling from 7.3% in 1995 to 4.9% in 1997.

A substantial increase in expenditure in 1997, which exceeded the total health budget, was primarily due to a large inflow of funds from vertical programs like the Global Fund (GFATM). This influx of donor funds was not captured in the regular budget process but was recorded in the accounts.

Impact of Decentralization on Financial Management

The decentralization of financial management has created complexities. The consolidated budget at federal and regional levels did not display the total intended sector allocations, as

lower levels of government (regions and Woredas) had the discretion to determine their own sector allocations. This has led to a situation where earmarked donor funding was often offset by a reduction in the federal subsidy to regions.

Furthermore, donor funding flows through three different channels, with Channel 3 funds often bypassing central and regional government financial controls, making them "off-budget" and "off-accounts". This lack of alignment and the low predictability of donor commitments undermine the integrity of the budget and public sector management.

Health Care Financing (HCF) Strategy and Progress

The HSDP II introduced a Health Care Financing strategy to increase resources, improve efficiency, and promote sustainability. Key reforms included user fee revision, revenue retention, and waiver and exemption systems. While the target was for 20% of government health expenditure to be covered by fees, preliminary data indicated that actual revenue collected was only about 12%. User fee revision was deferred due to concerns that it would be counter-productive without a consistent supply of pharmaceuticals, diagnostic capacity, and respectful staff.

The role of the private sector in health care has also increased, with private suppliers and public suppliers each accounting for 50% of services. The government has shown commitment to promoting the private health sector, and initiatives like the Revolving Drug Fund (RDF) and Special Pharmacies have been introduced to address chronic shortages of drugs.

Governance and Accountability

Governance in the health sector is highly decentralized, with the implementation of HSDP II being a joint responsibility of the Federal Ministry of Health (FMOH), Regional Health Bureaus (RHB), and Woreda Health Offices (WorHO). However, there was no direct accountability relationship between the FMOH and the RHBs or WorHOs; these lower-level offices are directly accountable to local governments. This "loose accountability" means the FMOH must use indirect methods such as dialogue and advocacy to influence decisions at the regional and Woreda levels.

It was also found that management capacity was weak at all levels, particularly at the Woreda level, due to a shortage of human resources and high staff turnover. The lack of knowledge about HSDP components and the Programme Implementation Manual (PIM) at the Woreda level was seen as a threat to effective implementation.

In conclusion, while HSDP II was an ambitious program, its implementation was hampered by severe under-funding, a lack of alignment and predictability in donor funding, and a fragmented governance structure. The success of future programs depends on strengthening financial

management, improving accountability, and enhancing management capacity at all levels, especially at the decentralized Woreda level.

Conclusion

The Health Sector Development Programme II (HSDP II) was a comprehensive three-year plan from 1995 to 1997 with a budget of 6.8 billion Birr, aimed at building upon the achievements of HSDP I and aligning with the Second National Development Plan and the Sustainable Development and Poverty Reduction Programme of the Ethiopian government. The program made notable progress in expanding health services and controlling major communicable diseases. The Health Service Extension Programme (HSEP), for instance, was considered a successful community-based initiative that focused on preventive, promotive, and curative services, particularly for mothers and children. Significant strides were also made in constructing new health facilities, with the number of health posts increasing by 2,901 and health centers by 192. The program successfully increased potential health service coverage from 61% to 72%. Furthermore, efforts to control diseases like malaria and tuberculosis showed positive results, including securing funding from the Global Fund and increasing DOTS coverage for TB to 90%.

However, the implementation of HSDP II was not without its challenges. The health sector faced significant under-funding, with actual expenditures amounting to only 43% of the estimated program costs. The government's health budget as a percentage of total public expenditure declined during this period. Human resource development lagged behind facility construction, leading to new health facilities remaining unstaffed for extended periods due to a critical shortage of skilled personnel and high staff turnover. The distribution of health professionals was also uneven, with most concentrated in urban areas. In the pharmaceutical sector, despite an increase in budget, challenges such as insufficient funding, regular stock-outs of essential drugs, and inadequate storage facilities persisted. Governance was another major constraint, characterized by a highly decentralized structure with "loose accountability" between the Federal Ministry of Health and lower-level offices, which often led to a lack of coordinated efforts. The lack of a clear strategy for integrating gender issues into health planning and for providing health care to the pastoralist population also highlighted areas needing improvement. The success of future programs will depend on addressing these systemic issues, including strengthening financial management, improving accountability, and enhancing management capacity at all levels.

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